

Ovarian cancer surgery

Surgery is one of the main treatments for ovarian cancer, fallopian tube cancer or primary peritoneal cancer. It also helps doctors to diagnose the type of cancer.

On this page

- [When is surgery used for cancer of the ovary, fallopian tube or peritoneum](#)
- [Surgery to remove the cancer](#)
- [Surgery if the cancer has spread](#)
- [Surgery to protect fertility \(fertility-sparing surgery\).](#)
- [How we can help](#)

When is surgery used for cancer of the ovary, fallopian tube or peritoneum

Surgery is one of the main treatments for [cancer of the ovary](#), [fallopian tube cancer](#) or [primary peritoneal cancer](#). You have surgery to remove the cancer. But it also helps doctors to:

- diagnose the [type of cancer](#)
- find out more about its [stage](#).

You usually have surgery to remove the following:

- the ovaries
- the fallopian tubes
- the womb (uterus)
- the layer of tissue that covers the stomach and part of the bowel (the omentum)
- lymph nodes.

If the cancer has spread to other areas in the pelvis, you may need more surgery.

We have more information on [the ovaries, fallopian tubes and peritoneum](#).

Side view ovarian with ureter

Image: The diagram shows a side view of a female body from the level of the belly button down to the upper thigh. It shows the structures and organs in the abdomen and pelvis. There are 3 small openings into the pelvis between the legs. The opening closest to the back of the body leads up into a tube-shaped space. This is the back passage (rectum). The opening closest to the front of the body leads up into a rounded hollow organ. This is the bladder. The opening in the middle leads up into a slightly larger hollow organ with thick walls. This is the womb (uterus). The womb sits behind and over the bladder. At the top of the womb, a thin short tube is shown leaving the womb. This is a fallopian tube. It leads to a small solid oval organ. This is an ovary. Above the bladder and womb, a large space is shown. This space fills the front half of the body. Most of the space contains an irregular bundle of thick tubing. This is the bowel. A long flap of tissue is shown hanging down in the space in front of the bowel. This is the omentum. A thin lining is shown covering the walls of the space. The lining also covers the surfaces of the bowel, bladder and womb. This lining is the peritoneum. A fine tube is shown behind the bowel and womb. This is a ureter. It enters the top of the image towards the back of the body. It travels down behind the bowel and womb and connects to the top of the bladder through a small opening.

Before your operation

Sometimes you have [chemotherapy](#) before surgery. This is to shrink the cancer. If the cancer comes back, it may be possible to have more surgery.

You may have chemotherapy into the tummy area (abdomen) during surgery. This is called hyperthermic intraperitoneal chemotherapy or [HIPEC](#).

If you have very early stage cancer and want to get pregnant in the future, it may be possible to have only the affected ovary and fallopian tube removed. This is called fertility-sparing surgery. A specialist doctor does this type of surgery. They are called a gynaecological oncologist.

Before you have surgery, your surgeon and specialist nurse will talk to you and answer any questions. It is important you understand what your operation may involve.

Surgery to remove the cancer

The surgeon aims to remove all the cancer or as much of it as possible. They usually remove the:

- ovaries and fallopian tubes – this is called a bilateral salpingo-oophorectomy or BSO
- womb and cervix – this is called a total hysterectomy
- omentum – this is called an omentectomy.

The surgeon will also check how far the cancer has spread. This is important because it tells them more about the [stage of the cancer](#). During surgery, they will:

- take samples of tissue ([biopsies](#)) from other areas nearby
- remove some [lymph nodes](#) in the tummy and pelvis
- put fluid into the tummy and collect it to test for cancer cells – this is called abdominal washing or peritoneal washing.

You may also need surgery to other areas, to remove as much of the cancer as possible. This depends on where the cancer has spread in the pelvis.

Early menopause

If you still have periods, having a total hysterectomy and BSO will cause your periods to stop (the menopause). This can cause [menopausal symptoms](#) such as hot flushes and vaginal dryness.

Surgery if the cancer has spread

Many cancers of the ovary, fallopian tube or peritoneum have spread when they are diagnosed. If the cancer has spread to other areas in the pelvis or tummy, it may not be possible to remove it all.

Your surgeon will try to remove as much of the cancer as they can. Doctors sometimes call this debulking or cytoreductive surgery.

If at the time, the surgeon does not think that surgery will lead to a better result, they will not go ahead. You will have different treatment, such as chemotherapy.

Removing part of the bowel

If the cancer has spread to the bowel, you may also need a section of bowel removed. If possible, the surgeon removes the affected piece of bowel and joins the 2 remaining pieces together. This is called a bowel anastomosis or rejoin.

Sometimes the surgeon cannot safely join the remaining pieces of bowel together. Instead, they bring the upper end of the bowel out onto the skin of the tummy. This is called a stoma. After the operation, you wear a bag over the stoma to collect stools (poo). A stoma may be temporary to protect the bowel rejoin. Or it may be permanent if a bowel rejoin is not possible.

If you are likely to need part of the bowel removed, your surgeon will talk to you about this before your operation. If you need a stoma, your hospital team and a stoma nurse will give you support and advice.

We have more information on [having a stoma](#).

Surgery to protect fertility (fertility-sparing surgery)

If you have very [early stage ovarian cancer](#), it may be possible to have surgery that means you can still get pregnant. This is usually possible if the cancer is stage 1a and low grade.

The surgeon only removes the affected ovary and fallopian tube. They leave the other ovary, other fallopian tube and the womb. During the operation, the surgeon checks the other ovary and may take biopsies from it.

If any biopsies, lymph nodes or fluid removed from the pelvis show the cancer has spread, you may need more surgery. This may include removing the womb, remaining ovary and remaining fallopian tube. You may find this difficult to cope with if you were hoping to get pregnant in the future.

Your specialist nurse will give you lots of support. They may be able to refer you to a counsellor for further emotional support. They may also refer you to a fertility specialist before surgery to talk about options.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Below is a sample of the sources used in our ovarian cancer information. If you would like more information about the sources we use, please contact us at informationproductionteam@macmillan.org.uk

Ovarian cancer: recognition and initial management. Clinical guideline [CG122] Published: 27 April 2011
Last updated: 02 October 2023 www.nice.org.uk/guidance/cg122 (accessed May 2024)

Ovarian cancer: identifying and managing familial and genetic risk. NICE guideline [NG241]. Published: 21 March 2024 www.nice.org.uk/guidance/ng241 (accessed May 2024)

A. González-Martín, P. Harter, A. Leary, D. Lorusso, R. E. Miller, B. Pothuri, I. Ray-Coquard, D. S. P. Tan, E. Bellet, A. Oaknin & J. A. Ledermann, on behalf of the ESMO Guidelines Committee. Newly diagnosed and relapsed epithelial ovarian cancer: ESMO Clinical Practice Guideline for diagnosis, treatment and follow-up. Published August 17, 2023. [www.annalsofoncology.org/article/S0923-7534\(23\)00797-4/fulltext](https://www.annalsofoncology.org/article/S0923-7534(23)00797-4/fulltext) (accessed May 2024)

[Cancer Information Development team](#)

Author

[Dr Alexandra Taylor](#)

Reviewer

Consultant Clinical Oncologist

[Royal Marsden Hospital, London](#)

Date reviewed

Reviewed: 01 March 2025 | Next review: 01 March 2029

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help